

MP-112: Hyperbaric Oxygen Therapy (HBOT)

Bellcourt Medical Policy | Version 1 | Status: ACTIVE | Effective for dates of service 2024-01-01 until superseded

Field	Value
Policy number	MP-112
Service code(s)	BHA-THER-1830
Applies to	All clients administered by Bellcourt unless the client's plan document or delegation addendum states otherwise
Owner	Clinical Policy Committee
Version	v1

1. Purpose and scope

This policy sets out the clinical criteria Bellcourt reviewers use to decide whether the service described in this policy (Hyperbaric Oxygen Therapy (HBOT)) is medically necessary. It is applied to prior authorization requests where the date of service falls within the effective period stated above. A client's plan document may exclude this service or impose additional requirements; plan provisions are checked first.

2. Definitions

Medically necessary: services that are appropriate and consistent with the diagnosis, in accordance with generally accepted standards of medical practice, and not primarily for the convenience of the member or provider.

3. Coverage criteria

The service is considered medically necessary when the criteria below are met, read together using the AND / OR connectors shown:

- 1 Diabetic lower-extremity wound of Wagner grade 3 or higher that has failed at least 30 days of standard wound care; OR
- 2 Soft-tissue or bone radiation necrosis.

4. Not medically necessary / limitations

- Diabetic wounds of Wagner grade 2 or lower.

Investigational indications

HBOT is considered experimental and investigational (see MP-117) and is excluded from coverage for the following indications:

- Autism spectrum disorder
- Sports injury recovery or athletic performance
- Long COVID

- Cognitive decline

5. Documentation requirements

Wound measurements, Wagner grade, wound-care log; radiation history if applicable.

If required documentation is missing, the request must be pended and the provider contacted for information. It must not be denied for lack of documentation until the information request has gone unanswered within the regulatory timeframe.

6. Version history

Version	Effective	Summary
v1	2024-01-01	Initial version.